

Question:
A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

- Answer Choices:
- A. Behavior therapy
 - B. Methimazole
 - C. Fluoxetine
 - D. Hearing aids

Reasoning + Answer:

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**Finding reasoning paths : **

- 1 . Short attention span , hyperactivity , impulsivity , and aggressive behavior -> Attention - Deficit/Hyperactivity Disorder (ADHD) -> Behavioral therapy
- 2 . Major depressive disorder in the family -> Potential psychological impact on the child -> Behavioral therapy
- 3 . History of expelled preschools and behavioral issues -> Possible ADHD or conduct disorder -> Behavioral therapy
- 4 . Short attention span and impulsivity -> ADHD -> Behavioral therapy

**Reasoning Process : **

- 1 . **Symptoms Analysis : **
 - The child exhibits short attention span , hyperactivity , impulsivity , and aggressive behavior . These symptoms are characteristic of ADHD , which is commonly diagnosed in children presenting with these behaviors .
 - The child frequently interrupts , is difficult to engage , and is very forgetful , which aligns with ADHD symptoms .
 - The aggressive behavior and difficulty following directions further support the possibility of ADHD .
- 2 . **Family History : **
 - The mother has major depressive disorder , which can have a psychological impact on a child , potentially exacerbating behavioral issues . However , this does not directly point to ADHD but could contribute to the child 's behavior .
- 3 . **Behavioral Issues : **
 - The child has a history of being expelled from preschools for hitting and disrupting classes , which suggests significant behavioral challenges .
 - These behaviors are consistent with ADHD , where children often struggle to control their impulses and follow rules .
- 4 . **Developmental History : **
 - The child was born at term and has met all developmental milestones , which makes a developmental delay or intellectual disability less likely .
 - The history of acute otitis media at age 2 could be incidental and unrelated to the current behavioral issues .
- 5 . **Treatment Consideration : **
 - For ADHD , the first-line treatment is often behavioral therapy , which helps children manage their symptoms and develop better coping strategies .
 - Medications like stimulants (e.g . , methylphenidate) are also used , but behavioral therapy is often the initial approach .

**Conclusion : **

The most appropriate next step in treatment for this 5-year-old boy , given his symptoms of short attention span , hyperactivity , impulsivity , and aggressive behavior , is behavioral therapy . This approach is consistent with the diagnosis of ADHD and aims to address the child 's behavioral challenges .

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<answer>
Behavior therapy
</answer>